

Contract Pharmacy Prevention of Duplicate Discounts Use with Contract Rx Carve Out Excel Audit Template

Purpose: The purpose of the Contract Pharmacy Audit is to improve contract pharmacies' compliance with 340B Program requirements. Covered entities remain responsible for the 340B drugs they purchase and dispense through a contract pharmacy. Covered entities are required to ensure ongoing compliance and the timely recognition of any 340B Program compliance problems at all contract pharmacy locations, while maintaining auditable records.

The contract pharmacy audit is divided into three compliance elements:

- **Contract Pharmacy Eligibility**
- **Contract Pharmacy Prevention of Diversion**
- **Contract Pharmacy Prevention of Duplicate Discounts**

Covered entities are encouraged to complete this tool quarterly for each contract pharmacy organization. Proceed through the steps as follows:

1. Identify the staff member to complete audit.
2. Identify and collect relevant data, as follows:
 - a. Covered entity's 340B contract pharmacy Medicaid billing policies and procedures
 - b. Contract pharmacy billing codes identifying Medicaid payers (e.g., BIN/PCN)
 - c. Contract pharmacy accumulation report from 340B split-billing software, if applicable
 - d. Contract pharmacy dispensing records for the most recent 3-month period
 - e. Communications with state Medicaid agency (if applicable)
 - f. Any agreements/arrangements with state Medicaid programs for contract pharmacies' billing with 340B drugs (if applicable)
 - g. Contract pharmacy carve-in listing on HRSA 340B OPAIS (if applicable)
<https://opanet.hrsa.gov/opa/CPSearch.aspx?SearchCarveIns>
3. Complete tables and answer corresponding assessment questions.
4. Complete the "Summary of Results."
 - a. This section is a brief summarizing statement of the self-audit results for senior leadership and other key 340B stakeholders.
5. Review the results with the 340B Steering Committee (or other compliance oversight committee as determined by entity's compliance program or policies and procedures).
 - a. Assess if the results are indicative of a material breach (refer to Establishing Material Breach Threshold Tool
<https://docs.340bpvp.com/documents/public/resourcecenter/establishing-material-breach-threshold.docx>)
6. Develop a corrective action plan, if applicable.
 - a. Attach corrective action plan that addresses the compliance issues identified in this self-audit.



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Covered Entity & Contract Pharmacy Information

1. Entity's Name	
2. Entity's 340B ID	
3. Name of the contract pharmacy organization(s) (not locations) being audited. (A contract pharmacy organization may be a chain or an independent pharmacy and have multiple service site addresses.)	
4. Date of the LAST self-audit.	
5. Audit sample period of THIS self-audit. (Note: First day of audit sample period should be the day after the last day of the previous audit sample.)	
6. Date of THIS self-audit.	
7. Name and title of individual completing THIS self-audit.	
8. Summary of results: Note areas for improvement identified.	
9. Actions to be taken:	

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Compliance Element: PREVENTION OF DUPLICATE DISCOUNTS

The covered entity's contract pharmacy is not billing Medicaid for 340B drugs dispensed unless the covered entity has an arrangement with the state Medicaid agency and the contract pharmacy is listed as "carve-in" in the HRSA 340B OPAIS.

Duplicate discounts are prohibited by section 340B(a)(5)(A) of the PHSA; that is, a drug purchase shall not be subject to both a discount under section 340B of the Public Health Service Act and a Medicaid rebate under section 1927 of the Social Security Act. Covered entities that use contract pharmacy arrangements are expected to ensure compliance with all the requirements in the Final Notice regarding Contract Pharmacy Services published at 75 Fed. Reg. 10272 (Mar. 5, 2010). Under those guidelines, contract pharmacies are prohibited from using 340B drugs to dispense Medicaid prescriptions and must carve-out unless the covered entity, the contract pharmacy, and the State Medicaid agency have established an arrangement to prevent duplicate discounts. The covered entity must report any such arrangement to HRSA.

CONTRACT PHARMACY CARVE-OUT MEDICAID BILLING ASSESSMENT

From the contract pharmacy dispensing records:

- Identify billing codes (e.g., BIN/PCN) used by the contract pharmacy to bill Medicaid claims.
- From the contract pharmacy dispensing records, select all prescriptions for the most recent 3-month period, including primary, secondary and tertiary billing codes.
- Sort report by billing codes.
- Verify that a Medicaid billing code is not a payer for any 340B drug.

Answer Assessment Question	Yes	No	N/A	Unsure
1) For contract pharmacies that carve out, is Medicaid <i>never</i> the payer for 340B dispensations?	☐	☐	☐	☐

Answer "N/A" to the question if all contract pharmacy locations carve in.

Answer "Yes" to the question only if Medicaid was not the payer for any contract pharmacy prescriptions in the sample period.

If response is "No" or "Unsure," explain:

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CONTRACT PHARMACY CARVE-IN VERIFICATION

Table 1

- List the name of each contract pharmacy location that carves in in column 1.
- Complete columns 2 and 3.
- Validate the contract pharmacy location's carve-in listing on the HRSA 340B OPAIS to complete column 4.

TABLE 1 Contract Pharmacy Carve-In Verification Table Date documented: _____ (Attach data to substantiate each contract pharmacy location)					
(1) Contract pharmacy name and store #	(2) State Medicaid billing policy available?		(3) Documented arrangement with all applicable state Medicaid agencies to prevent duplicate discounts?		(4) Listed with a "Carve-In Effective Date" in HRSA 340B OPAIS?
	YES	NO	YES	NO	YES NO

Table 1: Assessment Questions	Yes	No	N/A	Unsure
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1) For each contract pharmacy listed in Table 1, is the state Medicaid billing policy available?

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Answer "Yes" to the question only if all answers are "Yes" in the column titled "State Medicaid billing policy available" in Table 1.

If response is "No, or "Unsure," explain:

2) For each contract pharmacy listed in Table 1, does the covered entity maintain a documented arrangement to prevent duplicate discounts with all applicable state Medicaid agencies?

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Answer "N/A" to the question if all contract pharmacy locations carve out.

Answer "Yes" to the question only if all answers are "Yes" in the column titled "Documented arrangement with all applicable state Medicaid agencies to prevent duplicate discounts" in Table 1.

If response is "No," or "Unsure, explain:

3) For each contract pharmacy listed in Table 1, is a "Carve-In Effective Date" listed in the HRSA 340B OPAIS?

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Answer "N/A" to the question if all contract pharmacy locations carve out.

Answer "Yes" to the question only if all answers are "Yes" in the column titled "Listed with a Carve-In Effective Date in the HRSA 340B OPAIS" in Table 1.

If response is "No" or "Unsure" explain:

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CONTRACT PHARMACY CARVE-IN MEDICAID BILLING ASESMENT

Table 2

- Identify billing codes (e.g., BIN/PCN) used by the contract pharmacy to bill Medicaid claims.
- From the most recent 3-month contract pharmacy period of pharmacy dispensing records (including primary, secondary, and tertiary billing codes) randomly select 20 prescriptions for which Medicaid was the payer.
- Complete columns 1 through 5.
- Verify that the Medicaid carve-in practice matches the arrangement on file with HRSA and complete column 6.

TABLE 2

Contract Pharmacy Carve-in Medicaid Billing Assessment Table

Time period tested: begin date _____ to end date _____

(Attach actual data to substantiate Medicaid billing for each Rx#)

[illegible]

Table 2: Assessment Question	Yes	No	N/A	Unsure
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1) For each prescription tested in Table 2, is Medicaid billed according to state requirements?

Answer "N/A" if all contract pharmacy locations carve out.



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Answer "Yes" to the question only if all answers are "Yes" in the column titled
"Covered entity /state Medicaid carve-in practice matches documented
arrangement on file with HRSA" in Table 2.

If response "No" or "Unsure," explain: (Identify and discuss each outlier)